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Exploring the Impact of Culture and Acculturation on Older Families Caregiving for Persons With Developmental Disabilities*

Philip McCallion, Matthew Janicki, and Lucinda Grant-Griffin**

A series of focus group meetings were held with African American, Chinese American, Haitian American, Hispanic/Latino American, Korean American, and select Native American communities. Cultural issues in caring for persons with a developmental disability were discussed. Differences were found in the following themes: (1) how disability is perceived, (2) who are the family, (3) who provides care, (4) how the family makes decisions, (5) what family members expect of each other, (6) what support families receive from friends and community, (7) why the family moved, (8) cultural values important to family members, (9) family willingness to accept services from outside the family, (10) the family's first language, and (11) families' concerns about service providers. A range of adherence was observed as most prevalent among each of the cultures and across generations and age groups. Implications for the development and delivery of services are discussed.

Increasing attention has been paid to the needs of older families caring at home for family members with a developmental disability. Concerns have been raised that many families are not connected to services and have not planned for the future (Smith, Fullmer, & Tobin, 1994; Wood, 1993). Indeed, agency workers in both the aging and developmental disabilities service systems have reported that families are often resistant to their efforts, or that they themselves are unprepared for the issues that these families present (McCallion & Tobin, 1995; Sutton, Sterns, Schwartz, & Roberts, 1992). More limited attention has been paid to families from diverse cultures caring for family members with developmental disabilities. It is not known if their caregiving experiences are similar or different from other families, or whether prevalent emphases in the service system on permanency and future planning are appropriate or effective for families from diverse cultures. To address these questions, findings will be reported on a series of focus groups held with African American, Chinese American, Haitian American, Hispanic/Latino American, Korean American, and select Native American communities. The focus groups were held to gain an understanding of the uniqueness of the caregiving experiences of specific cultural groups, reasons for lower service use and the extent of cross-cutting concerns around aging, disability, caregiving and future planning. They were also a means to build trust, and to provide a context for future research.

Background

Informal caregiving is the primary mode of care in the U.S. for people who are aging, including those persons with a developmental disability (Doty, 1986; Seltzer, & Krauss, 1994). Although positive and rewarding aspects of providing care to a family member have been identified, stressful and burdensome aspects of caregiving have also been documented (Anthony-Bergstone, Zarit, & Gatz, 1988; Biegel, Sales, & Schulz, 1991; Seltzer & Krauss, 1994).

Key Concerns for Caregiving Families

Converging demographic trends are raising concerns both about the continued viability of relying on informal caregivers to meet long-term care needs and about the best ways to support families who are continuing to care for dependent adult family members (Doty, 1986). Those demographic trends include the growing numbers of persons who are aging, including persons with a developmental disability (Jacobson, Sutton, & Janicki, 1985; U.S. Senate, 1987-88). Also, increasing rates of divorce, geographic dispersal, and lower birth rates are reducing the number of potential family caregivers (Crystal, 1982). Furthermore, growing workforce participation and later child birth among the largest group of family caregivers, women, are increasing tensions between work, child care, and adult care responsibilities (Horowitz, 1985; Kiecolt-Glaser, Dura, Speicher, Trast, & Glaser, 1991; Lang & Brody, 1983).

Faced with such concerns, a primary focus of assistance by formal service agencies to caregivers of the elderly is the maintenance of family caregiving situations and the avoidance, or at least delay of institutionalization (for a review, see McCallion, Toseland, & Diehl, 1994). However, the focus of research and interventions for aging family caregivers of persons with a developmental disability has been somewhat different. Less attention has been paid to the maintenance of current caregiving and the avoidance of institutionalization. Indeed, many caregivers appear to be doing well and express little need for such assistance (Seltzer & Krauss, 1994; Smith, Fullmer, & Tobin, 1994). For these families, greater focus has been given to the development of plans for care when caregiving parents are no longer available, including arranging transitions to out-of-home placements (Heller & Factor, 1991; Smith & Tobin, 1993).

Lack of planning is a critical concern. Older families caring at home for adult sons and daughters with a developmental disability, routinely do not plan for the future. Also, many families actively avoid contact with service systems (Smith, Fullmer, & Tobin, 1994; Wood, 1993). The reasons for this include: a rejection of service systems because of poor experiences in the past; a belief that siblings will take responsibility for the person with a developmental disability, although this has rarely been discussed with them; a fear that planning will result in the immediate re-

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moval of the family member with a developmental disability; a lack of knowledge about service alternatives; a belief that no one else can provide the same level of care that they provide; and an unwillingness to acknowledge that through their own death or illness parents will fail to fulfill the promise they made to a son or daughter that they would provide them with lifetime care (Smith & Tobin, 1989; McCallion & Tobin, 1995).

When a parent dies or is hospitalized, lack of planning results in crisis-driven care decisions being made for persons with a developmental disability. Pressures of time and resources then make planning an exclusively staff process. The likelihood increases that people with developmental disabilities will be fitted into available (albeit, often inappropriate) placements. This is in conflict with prevailing cultural norms around self-determination and the preservation of independence. Use of services by these families has also not been planned for. The resources they will need have not been allocated (McCallion, 1993).

Families from Diverse Cultures

Little is known about the extent to which families from diverse cultures present the same or different needs and concerns. Although disabilities can and do occur in all cultural groups, studies of service provision nationally and in New York state indicate that persons of diverse cultures with a developmental disability, and their families, are under-represented among those enrolled for services with state developmental disabilities agencies (Grant-Griffin, 1995). For example, in a study comparing African-American and White family caregivers of persons with a developmental disability, Heller and Factor (1988) found that African American parents were somewhat less likely to use formal services and to plan for out-of-home placements.

Consistent with the changing demographics of the U.S., the number of caring families from diverse cultures is growing (Aponte & Crouch, 1995). If service connectedness and future planning is lower in diverse families of persons with developmental disabilities, one viewpoint is that the consequences from a lack of planning may be magnified. At the very least, the impact on future living arrangements is simply unknown. Regardless, the current and future needs of these families are even less likely to be planned for by service providers, particularly if their service needs are different from those identified for other groups.

Efforts to understand why service use is different have targeted understanding the influence of culture on service utilization. Culture represents the values, beliefs, customs, behaviors, structures, and identity by which a group of people define themselves (Axelson, 1993). Attention has also been increasingly focused upon understanding the process of acculturation. Acculturation is the process by which immigrants and other persons from non-majority cultures, willingly or unwillingly, give up ethnic values, customs, and behaviors for those of the majority culture (Aponte & Barnes, 1995). A number of views have emerged of the impact of culture and acculturation on the willingness of caregiving families from diverse cultures to utilize formal services.

Some investigations of the lower usage of formal services by members of cultural minorities have noted a greater reliance on filial piety, greater availability of extended family supports, suspicion of formal structures, and cultural beliefs that one should take care of one's own (Lockery, 1991; Sakauye, 1989). For example, Johnson (1995) points out that even for members of diverse cultures who appear to be, or who are assumed to be acculturated, the influence of past traditions can reduce use of services from which families would benefit. Johnson suggests that families from diverse cultures: (1) prefer family caregiving and not discussing their problems with strangers, (2) are suspicious and have low expectations of "White-culture" institutions, and (3) have difficulty justifying use of community services which are a modern intervention, and not part of their cultural traditions. This viewpoint has been used to support arguments that lower usage of formal services by members of diverse cultures may be their own choice, and it may be inappropriate and disrespectful to target resources to these populations.

This viewpoint has been challenged by a second perspective that there are greater dangers in stereotyping families by assuming that identification with a particular culture means a specific set of beliefs and practices (Gratton & Wilson, 1988). For example, some researchers (Miller, Campbell & Kaufman, 1996; Pruchno, Patrick & Burant, 1996) have compared African American and White caregivers. Both Miller and colleagues and Pruchno and colleagues concluded that race or culture, *per se*, are not sufficient to explain differences in caregiving experiences and service use.

A third perspective has emerged from this debate. Proponents agree that cultural values may discourage use of services. However, they do not require that all members of a cultural group experience their culture and caregiving in the same manner. Instead, three sets of mediators of service use by persons from diverse cultures have been identified. First, the impact of culture on caregiving and use of services is often changed by migration (Aponte, & Barnes, 1995). For example, McGoldrick's (1989) research with European immigrant communities, points out that the experience of migration forces families from a variety of cultures who value extended family caregiving patterns to regroup. Geographic distance means that there are fewer family members available, and regular contact between family members is reduced. Therefore, although maintaining a person with disabilities within the family may be the cultural norm, in the absence of available extended family members, there may be needs for future out-of-home placement.

Secondly, traditional values are often mediated by generation. Older generations, particularly those who emigrated late in life, tend to cling to values and traditions that they see as defining themselves. In contrast, first generation individuals born in the U.S. and those who emigrated while they were relatively young feel the most pressure to "fit in" so that they may support their families economically. They have been found to be more likely to acculturate and to adopt the values and practices of the dominant culture, even while claiming to retain their own cultural values. Second and third generations of immigrant families, meanwhile, often selectively reclaim traditions as a way of defining themselves, while resisting other traditions that they see as restrictive (Johnson, 1995; Lockery, 1991). Such differences cause tension between generations and may contribute to difficulties experienced in caregiving situations. They may also influence unwillingness to accept formal services, even when services are needed.

A third set of mediators for families from diverse cultures include historic discrimination for all, legal status concerns for some, and the foreignness of services which have been developed for, rather than by the communities to be served. Such factors are believed to make some families from diverse cultures wary of

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formal services and poorly informed of what is available. Rather than choosing not to use services, lack of knowledge and wariness may result in the most needy members of diverse cultural populations receiving the least amount of services (Gratton & Wilson, 1988; Lockery, 1991).

Migration, generation and other mediators of cultural influences are not limited to groups who enter the U.S. from other countries. The experiences of Native Americans and Americans of African and Hispanic/Latino heritage, as they migrate across the country from their regions of origin, may be similar to those described for immigrant cultures (Damron-Rodriguez, 1991; Lockery, 1991).

The relevance, prevalence and influence of culture, acculturation, and their mediators has not previously been examined among families caring for someone with a developmental disability. Understanding the concerns of caregiving families drawn from diverse cultures and successfully addressing those concerns requires systematic consideration of these issues. This will also be an important first step in the development and delivery of needed and welcomed services.

Research Questions

A study of the impact of culture and acculturation on caregiving experiences and its implications for formal service provision was undertaken using an opportunity sample drawn from members of six cultural groups living in New York State: African Americans, Chinese Americans, Haitian Americans, Hispanic/Latino Americans, Korean Americans, and Native Americans. The primary research question was: can cross-cutting culturally related themes be identified that will aid in the development of strategies to offer services to families from diverse cultures? The investigators had observed an increasing level of collective action by advocates from a number of different cultures which suggested that there may be commonalities to be built upon as well as differences to be considered.

Secondary research questions were: (1) Are focus groups an appropriate strategy to identify diverse cultural beliefs, attitudes and values? (2) Are important perceptions of disability, caregiving and aging tied to the values of specific cultures? (3) Are there identifiable parameters such as age and generation since migration that are associated with level of cultural adherence? and (4) Are services underutilized because the current service system and its future and permanency planning orientation is not offering what families from diverse cultures want?

Methodology

In 1993, the Administration on Aging funded a series of demonstration projects across the country to identify and demonstrate best practices for outreach and services for aging caregivers of persons with developmental disabilities (Janicki, McCallion, Force, Bishop, & LePore, 1996). This study was an outgrowth of one of these projects, and is a demonstration designed to identify and demonstrate best practices for outreach and services for aging caregivers of persons with a developmental disability. A cooperative effort among three state agencies, the New York State Office of Mental Retardation and Developmental Disabilities (OMRDD), the New York State Office for Aging, and the New York Developmental Disabilities Planning Council, it also included several university-based research and service centers.

One of the project's objectives was to demonstrate ways that area agencies on aging (AAAs) could locate and help older New Yorkers who are caring at home for an adult family member with a developmental disability (see Janicki et al., 1996). A second objective was to identify and implement outreach and service approaches likely to be successful in diverse communities. Activities related to this second objective were aided by supplementary funding from OMRDD's Multicultural Professional Development Institute (MPDI). MPDI staff were also active members of the research team.

Sample: Focus Group Participants

The various focus groups held as part of the data collection included family members involved in caring for an adult or child with a developmental disability, staff or administrators of community based services and cultural organizations, and leaders of the cultural community. Both immigrants and persons born in the U.S. were represented among families, staff, and community leaders in the Korean and Haitian groups. Mexican and Guatemalan participants were all born outside of the U.S. Family members for all groups ranged in age from 40 to 85. Approximately 80% of family informants were over age 50. In a small number of cases, more than one culture was represented in an individual family. For example, in one family the mother was Chinese American and the father was Hispanic/Latino American. Both male and female family members participated in the focus groups; two-thirds of the informants were women.

Data Collection and Analysis

A grounded theory approach (Gilgun, 1994; Glaser & Strauss, 1967) was taken by the investigators. The process began with the development of ideas and discussion points. These beginning points were systematically tested, expanded, modified and codified in a cumulative manner, using a constant comparative approach to the analysis of both tape recorded and written notes on successive focus groups. Work proceeded in four stages.

Stage one—development of initial questions. The investigators began with a consideration of the available literature on willingness and unwillingness to plan for the future, and on use and non-use of formal services in general, and developmental disabilities services in particular by members of cultural groups. Next, the senior investigator met informally with a convenience sample of 25 providers and parents of diverse cultures drawn from 300 persons participating in a statewide conference of families from diverse cultures and providers sponsored by MPDI. In these informal meetings the investigator tested the relevance of key issues emerging from the literature. As a result of the discussions, the list of key issues was added to and modified. From these initial efforts, a series of discussion points were developed (see Table 1).

These meetings were also used to identify the most appropriate data collection methods. A strong consensus emerged that focus groups were the appropriate starting point. Also, it was noted that membership in the focus groups should be heterogeneous rather than homogeneous; one community leader said, "Listen to our communities, don't divide us up."

Stage two—initial development of themes. Stage two was also facilitated by MPDI. Within two months of the completion of stage one, three two-hour focus group sessions were scheduled with a set of diverse cultural communities not participating in the

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Table 1
Focus Group Discussion Points

Overview of Cultural Beliefs
1. Who do you consider members of your family?
2. What are the different family roles and responsibilities?
3. How do families generally make decisions about important issues—who decides, who is part of the discussion?
4. How are the aged perceived?
5. How are persons with disabilities perceived?
6. What are traditional caregiving patterns towards the aged and toward persons with disabilities?
7. How is the formal service system perceived?
8. What informal supports are available within your community to caregiving families?
9. What is the role of religion in supporting caregiving families?
10. Are there differences and similarities across generations in attitudes toward and support of family caregiving?
Planning for the Future
1. What informal supports do families expect to rely upon?
2. What formal services do families believe they will need in the future?
3. What family needs are not currently being met through either formal services or informal supports?
4. Who is most effective in doing outreach to families in this community?
5. What are the most effective ways to help families learn about useful formal services that are available?
6. What are the most effective ways to encourage families to use available formal services?
7. What are the most effective ways to help families to plan for the future?

AoA project. Family caregivers, community leaders, and service providers were recruited from African American, Korean American, and Native American communities. The Korean participants were both immigrants and persons born in the U.S. The African American and Native American participants were born in the U.S., and multiple generations were represented. The groups varied in size from 6 to 15 members.

Discussion points were shared in advance of the group meeting. The senior investigator also met informally with members of each community in advance to answer procedural questions, and to gain an overall understanding of the community. English was not the first language of many of the participants in the Korean group. A member of their community, fluent in both English and Korean, was recruited as a co-leader. The focus group meetings for all three communities were structured around the provided questions, and open discussion was encouraged. Family caregivers were active participants in the discussions. Caregivers did not appear constrained by the presence of community leaders and service providers. With the permission of the participants, the meetings were tape-recorded, and notes were written immediately after the session. After listening to the tapes of the sessions and reviewing the notes, a number of cross-cutting key themes related to cultural and acculturation influences on aging family caregivers of persons with developmental disabilities were identified.

Stage three—confirmation of themes. A second series of group meetings were held with African American, Chinese American, and Haitian American groups who participated in the AoA project (Janicki et al., 1996). Again, family members, service providers, and community leaders were invited. Chinese and Haitian participants included both immigrants and persons born in the U.S. Multiple generations were represented. The group sizes ranged from 6 to 18 participants. The meetings were held as part of the project debriefing session at the end of each community's outreach efforts under the AoA demonstration project. The same set of discussion points was used. Interest was in finding to what extent the cross-cutting themes were confirmed, expanded, or contradicted in the discussions that ensued.

The third stage was concluded with a review of notes compiled by the senior investigator and other project staff, and a conciliation meeting involving project team members to review what was said and to determine the level of agreement on culture and acculturation themes. The second set of meetings and the subsequent review process appeared to confirm the importance and the completeness of the group of themes originally constructed.

Stage four—generalizability of themes. In the fourth stage, the lead investigator met with a large focus group of 35 participants including Hispanic/Latino (Puerto Rican, Mexican, and Guatemalan) and African American families, service providers, and community leaders. The group was held as part of a larger conference with an agenda to look at commonalities and differences among diverse cultures regarding caregiving and services for persons with a developmental disability. The group was conducted in both Spanish and English and involved communities different from those who had participated in the first three data collection stages. Again the themes appeared to be confirmed.

Results

Descriptors of cultural identity emerged from the focus groups. These included an expressed identification with one's cultural heritage, close involvement with at least one cultural organization, use of a language other than English, active practice of customs particular to a specific culture, use of services from organizations closely identified with one's culture, and concern for the preservation of cultural values which are either threatened, or not valued by the larger society. One's cultural identity and the experience of pursuing that identity within daily society did appear to greatly influence the experience of being a family that includes a member with a developmental disability. Of critical importance is that every family is unique; participants stressed this repeatedly.

In pursuit of the primary research question, a series of cultural themes were identified through the focus groups. The eleven themes identified were: (1) how disability is perceived, (2) who is the family, (3) who provides care, (4) how the family makes decisions, (5) what family members expect of each other, (6) what support families receive from friends and community, (7) why the family moved, (8) cultural values important to family members, (9) families' willingness to accept services from outside the family, (10) the family's primary language, and (11) families' concerns about service providers.

How Disability is Perceived

There are many different ways that families might view a disability, and the person who has a disability. Many families, service providers, and community leaders in the focus groups struggled not only with their own beliefs about disability, but also about how others perceived the disability and parents of children with disabilities. One mother said that her son who had a disability was God's special gift; another, from the same cultural group, saw her son as God's punishment. In some cultures, it was acknowledged that disability represents such a shame that they hide the person, to such an extent that it appears that there are no people with disabilities in that community. In other cultures, everyone in the community has "a soft spot" for persons with disabilities, and the person with a disability is an active participant in family and community life. The groups acknowledged that variation in how disability is perceived is likely to influence both

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the level of services needed and a family's willingness to accept those services.

Who are Family

Who makes up a particular family may not always be obvious to workers but may be very clear to members of the family. We often think of family as being those people who either live in the household, or individuals generally considered to be close blood relatives. However, the focus group members discussed how some households encompass three and four generations, including cousins, siblings, and in-laws, in the same household further expanding the number of available relatives who may provide care. In households encompassing one or two generations, some of those same relatives may be available locally, and for others, they may live at some distance and participate in caregiving in varying degrees. Other families may not have relatives available; even those living in the same household may not be willing to provide care. There are also some family members critical to caring and decision-making who workers might not immediately recognize as family members, for example: an individual who immigrated to the U.S. at the same time and from the same area; a sister-in-law from a marriage that is now over; a friend who has become part of the household. Others become involved only when they are needed.

The participants argued that extended and non-traditional family members must be neither assumed nor ignored. Finding out about such family members and understanding their roles will often require effort on the part of agency workers. One participant told of an African-American woman in New York city who was caring for her mother, her sister with a disability, and two children. The caregiver suddenly died. Various components of the service system placed the mother in a nursing home, her sister in a group home, and the two children in foster care. Meanwhile word-of-mouth had informed members of her extended family in South Carolina of her death. They in turn contacted a cousin in Chicago who agreed to assume the caring responsibilities, and who then moved to New York city to do so. It took her three months to reunite the traumatized family.

Who Provides Care

In every cultural group that participated in the focus groups, the primary caregiver was usually the mother of the person with the disability. However, participants also noted that other family members did provide significant levels of support and influenced decisions. For example, in multi-generational households it was not unusual for the majority of caring responsibility to have been transferred to a sibling or another younger relative, and yet the aging mother still received the respect of the primary caregiver. Participants also reported that grandparents and other family members often assumed caring responsibilities. Some participants explained that others in their households with chronic illnesses and related disabilities also needed care. In such households, the person with a developmental disability was often someone who provided care rather than someone who received it.

Participants pointed out that if workers focus exclusively on one or two family members, or make assumptions about roles and needs of family members, they risk disrupting important family relationships. Consequently, their help will be rejected, even by the family members they have targeted for help. This is illustrated by a story told of an African American grandmother caring for her grandson with a developmental disability because the boy's mother was being treated for a serious heart condition. The mother lived in another state and the worker had no contact with her. Given the grandmother's own health concerns, the worker began encouraging her to consider an out-of-home placement for the grandson. She also arranged for respite services and a day program for the boy. The grandmother abruptly withdrew her grandson from the day program and refused respite services. With great difficulty the worker finally got the grandmother to explain that everything she does for her grandson she discusses first with her daughter (the boy's mother). As soon as she raised the issue of out-of-home placement, her daughter told her to get the boy out of all services, otherwise she would come and care for her son herself. As the grandmother explained: "She's just upset, she can't come and take care of him. But if she's angry at me, we don't talk on the telephone everyday anymore. Some days she's the only person I talk to. And she may get sicker herself. I need the help, but not if it means hurting my daughter . . ."

How the Family Makes Decisions

Focus group members highlighted that families may make decisions about caring and other matters in ways that are different from the worker's own approach. For example, it was acknowledged that families often gave a lot of weight to a particular family member's opinion, such as a father or a grandparent, even when that opinion proved to be a barrier to services or future planning. Multi-cultural agency staff representatives participating in the groups pointed out that workers have to re-examine their beliefs and expectations about the caring roles of mothers and fathers, irrespective of whether both parents are present in the household or are continuing to participate in the life of the person with a disability. The same is true for other relatives. One agency worker reported how pleased she was about her work with one single Chinese mother caring at home for her daughter. After the daughter had been at home for some time with no day program, the worker had arranged for an evaluation for a supported employment program. Both the mother and the daughter were excited. Then the mother announced that she had changed her mind. There would be no evaluation. Later the mother explained to the worker that her own parents were opposed to the plan. The worker was surprised at the influence still accorded to the mother's own parents, individuals who were now in their eighties and living some distance from the caregiver's home. With further exploration, she discovered that for many years her parents had been the mother's only support, and, she was unwilling to go against their wishes. The worker suggested that the mother's parents be invited to a meeting so that the proposed program and other available options could be explained further. Over time they were persuaded to support consideration of a day program. Participants also pointed out that there were other families who were isolated from their extended families and communities and made all decisions alone.

What Family Members Expect of Each Other

All focus group members valued family support and expected family members to help with caring. However, the reality was quite different in individual families for many reasons. Native American families expected their other sons and daughters to take over caring, but acknowledged that economic realities forced those sons and daughters to leave the local area and traditional ways. Chinese American families talked of expectations of family support being compromised by acculturation of younger

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family members and reduced availability of extended family members. The parents of two adult children with severe mental retardation, and few available relatives lamented that no one was able to assist with care of their daughters or to take care of themselves as they aged. Haitian families talked about the importance of supporting other family members even in the face of acculturation and scattered family networks. Instances were described where services were refused because acceptance of outside help was seen as allowing other family members to shirk their family responsibilities. Workers need an understanding of need for family support, expectations of support, support actually received, and conflicts that result when expectations are unfulfilled.

Support Families Receive from Friends and Community

How families feel about receiving help from friends and neighbors varies widely by cultural group and by the individual family. How much help they receive also varies. Families among the focus group participants identified friends and neighbors who assisted them. In some cultural groups, help from churches and culturally based social and service organizations was also reported as particularly useful. As a Native American mother explained when talking about how she felt when her son with a disability was born: "I never felt alone. I always knew that there were people around who would help me."

Some caregivers expressed disappointment that more help was not received, particularly from churches. In contrast, other families did not seek and would not accept help from their community. Instead, they reported that they tried to hide the person with a disability because of the shame they felt. This was reported by participants from several of the cultural groups.

Why the Family Moved

Why do people come from another country to the U.S., why do people in small towns or rural areas move to large cities, and why do people living in urban areas move to the suburbs or rural areas? Reasons why a family with a member with a disability might move that were discussed in the focus groups included: to find greater tolerance, to escape shame and labeling, to find more appropriate services, or to "hide" the person with a disability. Focus group participants noted that understanding the reasons for a move can help to clarify a family's willingness to accept services, as well as their fears and concerns.

Cultural Values Importance to Family Members

Participants emphasized that the importance of cultural traditions and values should never be assumed for any family or family member. Strains related to migration and different generations were reported to affect the cultural values of people who enter the U.S. from other countries, as well as African Americans, and Native Americans who migrate across the country from their regions of origin. Adhering to traditional values was cited by others as a key reason for seeking and receiving informal family and community support. Adherence was also reported by some as presenting barriers to the acceptance of formal services, because those services are not part of the traditional way of doing things. In addition, some families reported conflicts and disappointments when adherence to cultural values varied among key family members. An older Haitian woman pointed out that the support that other family members would offer a mother caring for a son or daughter with a developmental disability represented something that was important about being Haitian. It demonstrated the importance and respect traditionally shown to older family members and to the idea of families caring for their own. A younger Haitian man agreed that he would always support his mother in her caring but said here in America the traditional ideas about respect for the authority of elders and the family were too restrictive.

A Chinese American leader explained that cultural adherence posed many dilemmas. He stated that every time a family accepts a formal service they and their culture are changed by it. Yet not accepting the service often places family members at a disadvantage, impacting the family's ability to compete in the work place. Other participants agreed and recommended that workers should seek to understand key values around aging, disability, and caregiving common to the culture with which the family identifies. They should then explore the family's level of adherence to those cultural values, and the conflicts and disappointments caused by variations in adherence among family members and key community members.

Families' Willingness to Accept Outside Services

During the focus groups most families who acknowledged a specific cultural identity expressed a preference to receive services within their own community. They cited the following advantages: reduced language and access problems, fewer fears of formal service providers, services were more familiar, and services offered were more likely to respect their traditions. Yet, some families more willingly accepted services from agencies not in their community. One woman said, "When I go to a local agency I hear them whispering about me. Strangers don't whisper. Or if they do, I don't care as much."

Still other families who "hid" their family member from their friends and neighbors found it helpful to deal with workers who understood their feelings and concerns. They also hoped that involvement of their own community's agencies would open up dialogues and greater acceptance within their community. For other participating family members, adherence to cultural values posed some difficult challenges, for example, a Korean father explained that he approached a service agency for assistance with caring for his daughter with a developmental disability, when he and his wife were both out at work. The agency offered a personal care attendant, but could not guarantee that the attendant would be Korean. He explained that it was not acceptable to him to have non-Koreans in his home. The agency's response was that this was the only service that they could offer him. He then refused the service and contacted his own parents still living in Korea. Since that time his father and mother have alternated coming from Korea at 6 month intervals each year to care for his daughter. Upon hearing this story, another Korean parent said that for her this was an old fashioned view and in some ways she preferred to receive services from outside her community.

Family's First Language

In addition to the barriers posed by workers' use of professional jargon and abbreviations, many families reported increased difficulty in accessing services because English is not their first language. Many informational materials are in English only, and many workers only speak English.

Many of the participating families reported knowing little about available services. Language difficulties, they said, exacer-

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bated the situation. Few service agencies have translator services available and neighbors, other family members, and even children are often relied upon as mediators for non-English speaking family members. This was a situation that many families stated they were not comfortable with, particularly since much personal information has to be shared. Language difficulties also create additional costs in accessing services. A Guatemalan mother who participated in the final focus group talked of her difficulty in accessing medical services for her daughter with a disability. The mother did not speak English, and the daughter was unable to advocate for herself. A relatively routine medical concern required six doctor visits before medical clinic staff understood what had to be done and took care of it. Each visit required a taxicab ride to the clinic. The mother pointed out that even free and low-cost services become expensive when these additional costs and the frustrations involved are added in. At the other end of the continuum, families with a good command of English complained of patronizing workers shielding them from information in the mistaken belief that they would not understand.

Family Concerns about Service Providers

All families expressed concerns about dealing with formal service providers. They reported concerns about whether they would provide the services they need; what the quality of those services would be; if they asked for one service, would they be required to accept other services they did not want; and would their asking for services be used to justify removing the person with a disability from the home. In addition, those group members who placed value on extended family and community supports, expressed that formal service providers are "strangers" and their use may even suggest failure. However, not all participants agreed with this contention, great variation in the strength of feelings on this issue was observed.

Focus group members expressed other concerns about formal service providers. For example, many reported that offices are not easily accessed, and that there had been instances of difficult or unsatisfactory interactions in the past between agencies and families. Participants reported feeling judged by professionals, particularly by professionals from another culture, no matter how supportive the worker was. Families were also concerned that information given to the agency would be used for other purposes. One example given was that all cultural groups believe that abuse of persons with disabilities is wrong, but all participants in the focus groups did not agree that all use of corporal punishment is abuse. Families expressed concerns that traditional practices in their homes may be misinterpreted and reported to police and social welfare agencies. Another concern expressed was that family living arrangements which include large numbers of people, perhaps involving several generations or other extended relationships, may be interpreted negatively by agency staff, resulting in reports to housing code agencies, and allegations of improper care. Use of formal agencies was also reported as attracting attention from other agencies such as the U.S. Immigration and Naturalization Service, independent of the family's actual legal status.

Secondary Research Questions

The secondary research questions were also addressed.

Use of focus groups. Participants were asked to rate the effectiveness of the focus group process. A number of the families commented on the benefits they felt from hearing that other families' experiences were similar. Service providers felt they got a better picture of family needs. Community leaders welcomed the open discussion of disability and family issues, and were most concerned about the challenges to traditional cultural values expressed during the sessions. One African-American mother commented, "I've never had an opportunity to talk like this and feel like other people are listening. Now it's your job [the investigators] to get everyone else to listen." There was general applause after she made this comment.

Some families who chose not to fill out questionnaires regarding other aspects of the study, were willing participants in the focus groups. They acknowledged their suspicions about how the questionnaires would be used, but felt that they could talk freely in the focus groups, and argued that the groups provided a truer picture of caregiving in their communities than any questionnaire.

Perceptions of disability, caregiving, and aging tied to specific cultures. The most powerful cultural values expressed were a desire to hide the person with a disability, families need to take care of their own, elders must be respected and supported, and services which respect and support cultural beliefs are most welcome. Chinese and Korean participants expressed a somewhat greater likelihood to do without services in the absence of respect for cultural values, Native Americans and African-Americans were less likely to hide the person with a disability, and Haitian and Chinese participants placed the greatest emphasis on respect for elders. There was great variation, however, within each culture.

Impact of age and generation since migration. A review of the notes and tapes indicated that younger participants and those generationally removed from the country or region of origin expressed less adherence to cultural values. Indeed they were more likely to express frustration both with values they found restrictive, and at the continued adherence to those values by others. For example, during one session, an older Haitian man began to explain the impact of Voodoo beliefs on why many Haitian families hid family members with disabilities. Younger participants interrupted to emphasize that these beliefs were not nearly as powerful as they once were.

When participants perceived a cultural value or belief as a strength, however, adherence did not vary as much across generations. A younger Native American mother explained that she had rebelled against her culture as a teenager and had only returned to the reserve after she was married. Since her son was born three years ago, she had received tremendous support from her extended family and neighbors. Now she found herself reconnecting with other aspects of her culture.

Permanent or future planning. For some of the families participating in the focus groups, permanent or future plans were not an issue. Family would take care of their own. For others, although it was not clear who would assume responsibility when the parents were no longer able, the idea of placing the family member with a disability in a living arrangement outside the family was unacceptable. A third group was willing to consider out-of-home placements but were frustrated by lack of placements, difficult interactions with agency personnel, and services being offered on the agency's terms. Again, younger participants and the furthest removed generations were more likely to be in the third group. Families in all three groups were willing to consider using some services. For example, a Hispanic/Latino mother who clearly identified herself with the first group was still interested

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in getting more information on financial benefits for which her son might be eligible.

Providers who participated in the focus groups emphasized that they better understood the needs of their communities and the cultural issues that influenced services. They stated that their staff were also more likely to speak other languages, and their agencies were more flexible in their hours of operation and in the range of supports they offered. Also, they were more likely to offer services that support family caregiving rather than emphasizing out-of-home placement. Focus group participants generally agreed with that service provider statement. However, among Korean and Chinese parents, in particular, there was a minority of parents who also wanted to access services outside their communities (a larger group would only consider Chinese or Korean services). They felt uncomfortable acknowledging their child's disability and their need for services in their own communities. Older African-American parents questioned why senior housing or housing for persons with disabilities would not accept them as families. A concern was raised in almost every group that when it came to planning, agencies only talk about individuals and, as a Hispanic/Latino community leader said, "For us, it's the family."

Discussion

This study addressed five research questions: (1) can cross-cutting culturally related themes be identified that will aid in the development of strategies to offer services to families from diverse cultures. (2) are focus groups an appropriate strategy to identify diverse cultural beliefs, attitudes and values, (3) are important perceptions of disability, caregiving and aging tied to the values of specific cultures, (4) are there identifiable parameters such as age and generation since migration that are associated with level of cultural adherence, and (5) are services underutilized and are few future plans made because the current service system and its future and permanency planning orientation is not offering what families from diverse cultures want.

Cross-cutting Themes

Through the structured process outlined in the methodology section, eleven cross-cutting cultural themes were identified that mediate family caregiving and willingness to accept outside services. Certainly, cultural barriers to accepting needed services are of concern. Such barriers were raised in the themes of why the family moved, willingness to accept services outside the family, the family's first language, and concerns about service providers. Language issues were among the most striking in this group. They are a barrier to gaining knowledge of available services, arriving at an understanding of needs, and building relationships between workers, families, and persons with disabilities. As Smith and Ryan (1987) argued, agency administrators and policy makers need to examine how to increase access to workers who speak a family's primary language or to extensive, appropriate translator services.

The focus group participants identified two other groups of pertinent cultural themes. The first was a group of core cultural values around what is disability, who is the family, and the cultural values that the family identifies as important. Where perceptions of disability cause families to "hide" the person with a disability, willingness to accept services appeared lowest. The perception of disability did not appear to be mediated by generation or by migration to the same extent as other values. When seeking to understand caregiving experiences and willingness to accept services for families from diverse cultures caring for someone with a developmental disability, the perception of disability appears to be a critical value to consider. Participants also indicated that important family operations are culturally driven—who provides care, how the family makes decisions, what family members expect from one another, and what supports families receive from friends and community. Failure by workers to understand and work within ascribed cultural roles in decision-making was frequently highlighted as a major barrier to accepting services.

Many workers entered the fields of aging, developmental disabilities, or other human service concerns because they wanted to work with people, and because they hoped to make a positive difference in people's lives. Indeed, as agency workers we often have information or access to resources that enables us to make that difference. Workers are often surprised, therefore, that their help is rejected when reaching out to older families caring for someone with a developmental disability, or families are fearful or suspicious of their intentions. It is important that workers be willing to reflect upon what they are doing or not doing to understand the extent to which they are contributing to that rejection. Consideration of the cross-cutting cultural themes provides a structured, effective means to do this.

Some families will openly discuss cultural and family issues. For others, it may take the worker some time to build up levels of trust sufficiently to achieve this. Focus group participants recommended that workers first be open to family structures, and ways of caring that are different from their own. Equally important, workers should avoid stereotyping family members by virtue of their gender or by society's expectations. Workers must also be open to recognizing that different values and structures may mean that a different range of services are called for. The cross-cutting cultural themes identified through this project are based upon these principles. They are offered as a means for workers to understand how culture and acculturation may impact a particular family and its caregiving and as a first step in planning for truly individualized services.

Use of Focus Groups

Using a focus group approach proved effective and appropriate. A variety of diverse communities responded positively to the opportunity to participate, and there was representation from caregiving families, cultural agency staff, and community leaders from each community. Participants stated that they were pleased to have an opportunity to have their voices heard and to shape services for their communities.

It is usually recommended that membership of focus groups be homogenous rather than heterogeneous (Krueger, 1994). That recommendation was clearly violated in this study by the inclusion of family members, service providers, and community leaders in one group. As the participants themselves pointed out, combining the groups meant that a fuller community picture emerged, and it was an approach with which they were more comfortable. Also, the group composition contributed to opening up discussions around disability issues in some communities where such discussions rarely occur. Nevertheless, there was an on-going concern on the part of the investigators that families might not speak up in such a forum. That did not appear to be the case. One mother explained that she was not going to miss her opportunity to finally get people to listen.

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Of more concern was whether or not the approach allowed for full discussion of specific cultural topics. The initial meetings with parents and providers encouraged the development of a number of questions regarding the role of churches and religiosity. Yet themes around these issues did not emerge from the focus groups. Many families stated that they held religious beliefs, and some talked about the support they received from churches. However, these beliefs and practices were not presented as a major part of the experience of caregiving in their cultures. As was reported earlier, there was an exchange between older and younger Haitians on the impact of religious beliefs in which the younger members quickly interrupted and contradicted the views being expressed. This may illustrate that there was not the same comfort level among participants when they discussed religion, as was true for other areas. This deserves further exploration with other groups and in other locations. Also, other research approaches may prove more fruitful in gathering information on this topic and may also help determine if there were other cultural topics that were not fully discussed because of the focus group make-up and approach.

Uniqueness of Cultural Beliefs

The findings outlined in the results section demonstrate that participants in the focus groups did believe that perceptions of disability, caregiving, and aging are closely tied to their cultural values. The findings did not support that there is a reliable cultural profile to which members of a specific cultural group may be expected to conform. Similarities between cultures, such as the importance accorded the family and support for elders, were most striking. Differences were reported in the level of emphasis on these values. Rather than a fixed pattern of cultural beliefs, we found that each group described a range of adherence found in their community, and emphasized the importance of exploring and respecting that range. Indeed the cross-cutting cultural themes were recommended as a starting point for such explorations.

Impact of Age and Generation

There was less cultural adherence, or it was achieved with greater difficulty, among the younger participants, and among those participants who were more generationally removed from their country and region of origin. However, we also observed that a small number of new immigrant Haitians and Koreans were more focused on the benefits of being in the U.S., and were less attached to their original cultural beliefs. In addition, culture of origin was valued among younger and generationally removed participants. As Johnson (1995) has also noted, later generations and younger participants did identify cultural values to which they adhered, or at least respected. For example, one African-American provider said that she was "not a stereotype," but "don't tell me that the things my parents and grandparents believed in are not relevant and a part of my day-to-day life."

Age and generation did provide some clues about likelihood of acculturation and of cultural adherence. Our findings suggest, however, that actual levels of adherence need to be determined on an individual basis before conclusions are drawn about need for and willingness to accept services. Again, consideration of the cultural themes offers a means to assess the level of importance and the extent of adherence attached to specific values.

Future Planning

Not all families in the focus groups were opposed to future planning or even to out-of-home placements; although many were. The availability of other family members, fears for the future, and perceptions of either declining or continuing adherence by others to cultural values were important mediators. What did emerge was that many families, service providers, and community leaders in the focus groups did not perceive that workers presented permanent and future planning in ways that could be easily reconciled with their cultural values. Participants reported that services were often offered as replacements rather than supports, focused on moving people into out-of-home situations rather than maintaining households, were individual rather than family-oriented, and implied failure rather than built on strengths. In all of the groups, we observed that the respondents felt that the best interests of the person with a developmental disability would be better served through the maintenance of overall supports to the whole family. The lesson learned was that workers who hope to be successful in aiding families from these cultural backgrounds need to have an appreciation that maintenance of family caring is likely to be preferable to out-of-home placement.

The view of future planning that emerged from the focus groups challenges existing emphases on planning for independence and future transitions for the individual with a developmental disability. Openness from the worker and from agencies to consider supporting continued family caring will increase willingness in communities to accept needed services. Agencies and workers will struggle with balancing increasing access to services and ensuring the protection of the best interests of the person with a developmental disability. One tool workers may use to reconcile these concerns is to consider the importance of the family and of cultural values for the person with a developmental disability.

Conclusions

Previously established principles for good practice with older caregiving families suggest that workers should begin by developing both an understanding of each family's caregiving experience, and a collaborative alliance with all interested members of the family (McCallion & Toseland, 1993). For families from diverse cultures, this is particularly true. One's cultural identity and the experience of pursuing that identity within daily society greatly influence being a family that includes a member with a developmental disability. The cultural themes identified through this project offer workers a means to assess the role of culture, family by family. Exploration of the themes coupled with consideration of one's specific culture, age, and generational distance from country or region of origin will also help workers to avoid falling prey to cultural stereotypes. More work is recommended, however, in pursuing greater understanding of the impact of culture and acculturation on caregiving and their impact upon service need and utilization.

The focus group findings answer affirmatively that families from diverse cultures do present a caregiving picture different from that on which current services are based. For service agency administrators and policy makers, the focus groups findings are a challenge to re-examine policy and service approaches that focus on individuals rather than families, and which encourage out-of-home placement. Tendencies for greater diversity in cultural adherence were identified among younger members of communities

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and among those generationally distant from country and region of origin. This further encourages openness to a range of services, rather than assuming that one set of programs will serve all.

Administrators and policy makers must also reconsider whether current agency networks are sufficient or appropriate to reach out to and serve families from diverse cultures. For example, agencies lacking staff who speak the family's primary language proved to be an important barrier. Also, many families in the focus groups expressed greater interest in receiving services from agencies based in and operated by their own multi-cultural communities. Current agency networks rarely include such agencies. In a time of declining resources, there is less inclination to consider expanding the number of providers among whom resources will be shared. However, the groups we spoke with identified services they needed and were not receiving. Greater inclusions of multi-cultural agencies as part of the services network can work to expand access, ensure that needed services are delivered, and help in creating service models and approaches that better meet the needs of persons from diverse cultural backgrounds.

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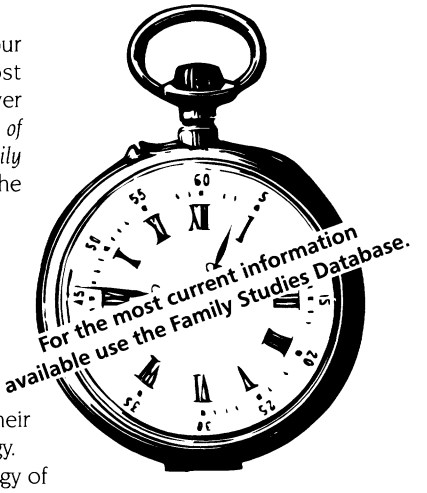

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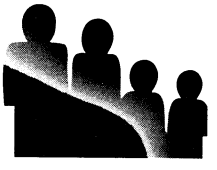

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